NCCP 00361 · VERSION 4

Fulvestrant · injections every 4 weeks

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Fulvestrant: material risks

☐ Injection-site pain and uncommon nerve injury

☐ Joint / muscle pain

☐ Bleeding / bruising risk with intramuscular injections

☐ Hot flushes, fatigue and nausea

☐ Liver enzyme changes or liver injury

☐ Blood clots and hypersensitivity reactions

Discuss the treatment aim, injection discomfort and possible nerve injury, bleeding or bruising, liver effects, flushes, joint symptoms and fatigue. Liver impairment or an existing modified dose requires an agreed prescriber and pharmacy plan. Where relevant, use effective contraception during treatment and for two years after the final dose under the EMA product information. Pregnancy and breastfeeding are excluded. Tell staff about blood thinners; do not stop them yourself.

Individual benefit, alternatives and concerns

Agreed checks and next review

Before treatment, check blood count, kidney and liver function, the menopausal and treatment history, pregnancy exclusion and bleeding risk. Before injections, tell staff about anticoagulants, bruising, bleeding or previous nerve symptoms. Repeat liver tests when indicated; scans follow the agreed cancer-control plan. Tell anyone ordering an oestrogen blood test that you receive fulvestrant because some tests can give a misleadingly high result.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.